

Frequently Asked Questions – Cyclosporiasis

Audience: Physicians, advanced practice providers and other all other clinicians

Purpose: To provide clinical guidance on the presentation, diagnosis and management of cyclosporiasis

Last updated: July 28, 2026

What is cyclosporiasis?

- Cyclosporiasis is an intestinal illness caused by the coccidian parasite **Cyclospora cayetanensis**.
- Humans are the only known hosts.
- Cases typically rise in the summer months.
 - CDC considers May 1 to August 31 the annual cyclosporiasis season.

How is it transmitted?

- It is transmitted via the fecal-oral route through ingestion of **sporulated oocysts** in contaminated food or water.
 - Consumption of contaminated food such as fresh produce (sources in current and prior outbreaks) or water is the most common route of infection.
- No direct person-to-person transmission.
 - Cyclospora oocysts are excreted unsporulated and require days to weeks in the environment to sporulate and become infectious.

What is the state and source of the current outbreak?

- As of 7/28/26, at least 4173 cases have been reported in 41 states.

- 408 hospitalizations and zero deaths.
- **No confirmed cases in California, to date, from this outbreak.**
- Source is thought to be iceberg lettuce from Taylor Farms de Mexico.
 - Products were sold under multiple different names in 27 states.
- Previous outbreaks in the U.S. have been linked to imported fresh produce, including cilantro from Mexico, berries, and prepackaged salad mixes.

What are the symptoms?

- The average incubation period is approximately **7 days** (the range 2-14 days).
- Symptoms include:
 - Watery, sometimes explosive, foul-smelling diarrhea (median 6 stools/day).
 - Loss of appetite and weight loss (median 3.6 kg).
 - Fatigue, abdominal cramping, bloating, and flatulence.
 - Nausea, low-grade fever, myalgias, and headache.
 - Symptoms characteristically **wax and wane** and can persist for days to months if untreated.
 - Severe disease is uncommon and more likely in immunocompromised individuals.

When should clinicians consider testing for cyclosporiasis?

- Immunocompetent patients with persistent diarrhea (>14 days).
- Returning travelers with persistent diarrhea from areas where current outbreak is happening (>14 days).
- Consider for immunocompromised patients with diarrhea.

What tests should be ordered if you suspect Cyclospora?

- Routine ova and parasite testing **does not reliably detect Cyclospora oocysts.**
 - Specific testing must be explicitly requested.
- Recommended Test: CRYPTOSPORIDIUM, CYCLOSPORA, AND CYSTOISOSPORA SMEAR
 - Health Connect order number 87207U.
 - The oocysts are shed intermittently, so multiple specimens may need to be ordered.
 - Test TAT 2-4 days.
- Order other stool tests, as clinically indicated
 - C diff, O&P
- **Serologic testing is not available.**
- Cyclosporiasis is a reportable disease in California.
 - Confirmed cases will be automatically reported via processes KP already has in place.

What is the best treatment for cyclosporiasis?

- Advise patients to stay well hydrated and replace electrolytes.
 - **Most immunocompetent patients recover without treatment, but it can take months for a full recovery without treatment.**
 - Consider treatment if:
 - Symptoms are severe, prolonged or relapsing.
 - Patient is severely immunocompromised, such as:
 - Actively receiving treatment for cancer.
 - Receiving immunomodulating treatments.
 - Untreated HIV.
- **Treatment:**
- Adults: Trimethoprim-Sulfamethoxazole (TMP-SMX or Bactrim) 1 DS tablet twice a day for 7 days.
 - Pediatric: (2 months or older- 18 years): 8-10 mg/kg TMP and 40-50 mg/kg SMX per day orally in 2 divided doses for 7 days.

- If patient has a sulfa allergy, please send a Dr. Advice to ID to discuss the next steps.

What is the management in immunocompromised/ pregnant/lactating individuals?

- Patients diagnosed with cyclosporiasis who are immunocompromised or cannot receive Bactrim should be discussed with your local ID physician.

What advice should I give patients to prevent cyclosporiasis?

- Stay up to date on food safety alerts and do not eat recalled food.
- **Wash:** Wash hands with soap and warm water before and after handling or preparing fruits and vegetables.
- **Prepare:** Wash all fruits and vegetables, even if labeled “prewashed”, thoroughly under running water before eating, cutting, or cooking. Scrub firm fruits and vegetables, such as melons and cucumbers, with a clean produce brush. Cut away any damaged or bruised areas on fruits and vegetables before preparing and eating.
- **Store:** Refrigerate cut, peeled, or cooked fruits and vegetables as soon as possible, or within 2 hours.

What infection prevention/control standards are needed in the healthcare setting?

- Cyclosporiasis is not usually spread directly from person to person.
 - Cyclospora oocysts require time in the environment to become infectious.
- Clinicians should follow **Standard Precautions**, including gloves when direct contact with feces or potentially contaminated materials is anticipated.
 - Use a gown, facemask, and eye protection or face shield when splashing or spraying may occur.

- Perform hand hygiene before and after every patient contact. If hands are visibly soiled or after direct contact with fecal material, wash hands with soap and water.
- For patients with gastroenteritis who are diapered or incontinent of stool, use **Contact Precautions** when providing direct patient care.
- For environmental cleaning, remove visible soil first, then apply a hospital disinfectant according to product instructions for use.
 - Staff should remove gloves after cleaning and perform hand hygiene.

References:

1. [Cyclospora Outbreak Linked to Iceberg Lettuce in 9 States | Cyclosporiasis | CDC](#)
2. [Investigation Update: Cyclospora Outbreak, July 2026 | Cyclosporiasis | CDC](#)
3. [California Not Among States Experiencing Increased Cyclosporiasis Cases](#)
4. [Cyclosporiasis](#): cdph.ca.gov
5. Human Cyclosporiasis - ***Lancet Infect Dis 2019; 19: e226–36***; March 15, 2019
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